

Media Information

From: Main Street Radiology To: ALICE Page: 1/2 Date: 9/16/2025 9:18:42 AM

Tel (718) 428-1500 • Fax (718) 428-2475
www.mainstreetradiology.com

Bayside 32nd Ave	33-25 Francis Lewis Blvd, Bayside, NY 11358
Bayside 44th Ave	44-01 Francis Lewis Blvd, Bayside, NY 11361
Downtown Flushing	136-25 37th Ave, Flushing, NY 11354
Western Queens	72-06 Northern Blvd, Jackson Hts, NY 11372
Glendale	83-14 Cooper Ave, Glendale, NY 11385

: HELEN HSIEH, MD
: 133-47 SANFORD AVE, SUITE 1B
FLUSHING, NY [REDACTED]

Referrer Phone: (718)661-6630
Name: WEN MEI I YEH
Phone: [REDACTED]
Gender: Female
DOB: [REDACTED]
Patient: [REDACTED]
Exam Date: 2/5/2025 11:06 AM

Acc #: 9695619
Secondary ID: 191758
Exam Name: [REDACTED]

Mammogram:

Clinical information: Patient [REDACTED] screening. The patient [REDACTED] of left core needle biopsy in 2009 - benign. The patient [REDACTED] The patient [REDACTED] sister, at age 60, breast cancer.

Breast Cancer Risk:
NCI 5 year:2.8%
NCI Lifetime:7.9%

Last reported clinical breast exam: Within the past year

Comparison: [REDACTED] (mammogram), [REDACTED] (mammogram), [REDACTED] (mammogram) and [REDACTED] (mammogram).

3D digital CC and MLO views of both breasts were obtained. Reconstructed CC and MLO views were obtained. Deep learning algorithms (AI) were applied to mammography images and DBT services as a concurrent reading aid.

The breast tissue is heterogeneously dense, which may obscure small masses. A few benign type calcifications are noted including faint vascular calcifications. There is an asymmetry with possible distortion in the lateral right breast in the CC image. Scarring is noted in the lateral left breast. No [REDACTED] distortion are identified on the left.

Bilateral Breast Ultrasound:

Comparison: [REDACTED] and 1/27/2023

Technique: Sonographic evaluation of both breasts in their entirety was performed, including each of the four quadrants, the retroareolar region, and the axilla, in clockwise fashion.

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Main Street Radiology

Name: [REDACTED]
Date of Birth: [REDACTED]

Right Breast:

No [REDACTED]

Left Breast:

No [REDACTED]

-3:00, 3 to 4 cm from the nipple, scarring is noted.

-5:00, 2 cm from nipple, there is a 0.3 cm cyst and a shadowing calcification, benign.

IMPRESSION:

1. Right breast asymmetry with possible distortion. Further evaluation with spot compression imaging and targeted ultrasound is recommended.

2. No [REDACTED]

2. No [REDACTED]

BI-RADS CATEGORY: 0 - Incomplete - Need Additional Imaging Evaluation

Recommendation: Diagnostic right breast mammogram and ultrasound.

A letter will be sent to the patient [REDACTED]

Approximately 10% of breast carcinomas are not radiographically detectable. A negative report should not delay biopsy if a clinically suspicious mass is present. Dense breasts may obscure an underlying neoplasm.

Patient [REDACTED] a target due date for their next mammogram.

BIRADS 1: Negative

BIRADS 2: Benign

BIRADS 3: Probably Benign

BIRADS 4: Suspicious Abnormality - Biopsy should be considered

BIRADS 5: Highly Suspicious of Malignancy

BIRADS 6: Known Malignancy

BIRADS 0: Incomplete - Need Additional Imaging Evaluation and/or Prior Mammograms for Comparison

The New York State Department of Health requires us to indicate the date of the patient's last clinical breast examination.

Electronically signed by: Allison Rubin MD [REDACTED] 8:50 AM

Workstation: NYPQARUBIN

Report Electronically Signed by: Allison Rubin MD
Report Electronically Signed on: [REDACTED] 08:50 AM

Document Information

Misc Administration: Outside Radiology, Breast

MA 3D DIGITAL SCREENING MAMMO

00:00

Attached To:

RADIOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Priscilla P. Hazel | Msk Pas